

August 20, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Renato Dare (MRN MUSC2558722), Claim MUSC-FFD5-4

Patient	Renato Dare (DOB 1976-06-14)
MRN	MUSC2558722
Member ID / Group	SCD346795729 / GRP-86928
Claim number	MUSC-FFD5-4
Date of service	2026-06-22
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$490.01
Denial code	CARC 11 / RARC M64 — The diagnosis is inconsistent with the procedure.
Appeal deadline	2026-09-17

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this formal first-level appeal on behalf of patient Renato Dare (MRN: MUSC2558722, Member ID: SCD346795729) regarding the denial of Claim MUSC-FFD5-4 for services rendered on June 22, 2026, by Sarah E. Whitmore, MD (NPI 1447382910). South Carolina Medicaid (Healthy Connections / SCDHHS) denied the claim in full on August 18, 2026, citing CARC 11 ("The diagnosis is inconsistent with the procedure") and RARC M64 ("Missing/incomplete/invalid other diagnosis"), with the payer remark stating: "The diagnosis code reported does not support the procedure billed." The billed amount is \$490.01, with an allowed amount of \$367.12 and zero payment issued. MUSC Health respectfully contends that this denial is not supported by the clinical record and requests reversal and payment at the allowed amount.

CLINICAL BACKGROUND

Mr. Dare is a 50-year-old male established patient with a documented, active chronic disease burden that includes type 2 diabetes with neuropathy, prediabetes, metabolic syndrome, hypertriglyceridemia, hyperglycemia, and anemia, all of which are recorded in his longitudinal medical record. He presented on June 22, 2026, at MUSC Health Florence Medical Center, an on-campus outpatient hospital facility (Place of Service 22). The encounter was billed as CPT 99214, an established patient office or outpatient visit of moderate complexity, with a primary diagnosis of J06.9 (Acute upper respiratory infection, unspecified) and a secondary diagnosis of R69. Mr. Dare has a prior history of acute viral pharyngitis and viral sinusitis, both coded to J06.9, and his active comorbidities — including diabetes with neuropathy and metabolic syndrome — are well established in the record and directly relevant to the complexity of any acute illness evaluation in this patient.

BASIS FOR APPEAL

The denial under CARC 11 and RARC M64 appears to rest on the premise that J06.9 alone is insufficient to support a level-4 established patient visit. This position does not accurately reflect the totality of the clinical record. CPT 99214 is appropriately assigned when the medical decision-making or total time meets the threshold for moderate complexity. For an established patient carrying active diagnoses of type 2 diabetes with neuropathy, metabolic syndrome, hypertriglyceridemia, hyperglycemia, and anemia, an acute upper respiratory illness represents a materially more complex clinical evaluation than it would for a patient without such comorbidities. The management of an acute infection in the context of these active chronic conditions — including consideration of medication interactions with his current regimen of extended-release metformin and diphenhydramine — reasonably supports moderate-complexity decision-making.

Furthermore, the secondary diagnosis code R69 was reported on the claim and is associated in this patient's record with active conditions including hyperglycemia, metabolic syndrome, and hypertriglyceridemia. The payer's assertion that the other diagnosis is missing, incomplete, or invalid is inconsistent with the submitted claim data, which included R69 as a secondary diagnosis. To the extent the payer requires additional specificity, the clinical record supports the use of more granular codes for the active chronic conditions, and MUSC Health is prepared to submit a corrected claim with expanded secondary diagnosis coding if the payer determines that would resolve the edit.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) reverse the denial of Claim MUSC-FFD5-4 and issue payment at the previously established allowed amount of \$367.12 for CPT 99214 rendered on June 22, 2026. In the alternative, if the payer's clinical review team determines that additional diagnosis specificity is required to satisfy the coding edit, MUSC Health requests written guidance identifying the specific codes needed so that a corrected claim may be submitted without delay. This appeal is filed within the 30-day deadline established in the denial notice dated August 18, 2026, and is submitted in accordance with the Division of Appeals and Hearings' accepted procedures.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record